

DISCHARGE SUMMARY

Riverside General Hospital · Internal Medicine Service

PATIENT DEMOGRAPHICS

Patient ID: P456
Name: John A. Doe
Date of Birth: 1962-03-14 (Age 64)
Sex: Male
MRN: 884201
Attending Physician: Dr. Sarah Chen, MD (Internal Medicine)
Admission Date: 2026-04-12
Discharge Date: 2026-04-18
Length of Stay: 6 days

CHIEF COMPLAINT

Substernal chest discomfort and shortness of breath on exertion for the past 5 days, worsening over the 24 hours preceding admission.

HISTORY OF PRESENT ILLNESS

Mr. Doe is a 64-year-old male with a history of poorly controlled type 2 diabetes mellitus, essential hypertension, and hyperlipidemia, who presented to the emergency department on April 12, 2026 with progressive substernal chest pressure radiating to the left arm. The discomfort was associated with diaphoresis, mild nausea, and exertional dyspnea. ECG on arrival demonstrated 1.5 mm ST-segment depression in leads V4 through V6. Initial high-sensitivity troponin I was elevated at 0.42 ng/mL (reference less than 0.04). The patient was admitted to the cardiac care unit for management of non-ST-elevation myocardial infarction (NSTEMI).

PAST MEDICAL HISTORY

1. Type 2 diabetes mellitus, diagnosed 2018, last HbA1c 8.4%.
2. Essential hypertension, on lisinopril for 9 years.
3. Hyperlipidemia, on atorvastatin since 2021.
4. Obesity (BMI 31.2).
5. Former smoker, 20 pack-year history, quit in 2019.
6. No prior myocardial infarction or stroke.

ALLERGIES

Penicillin (rash). Sulfa drugs (rash, mild facial swelling). No known food allergies.

HOME MEDICATIONS ON ADMISSION

- Metformin 1000 mg PO twice daily
- Lisinopril 20 mg PO daily

- Atorvastatin 40 mg PO nightly
- Aspirin 81 mg PO daily
- Vitamin D3 2000 IU PO daily

HOSPITAL COURSE

The patient was started on dual antiplatelet therapy with aspirin 81 mg and clopidogrel 75 mg, anticoagulation with enoxaparin 80 mg subcutaneously every 12 hours, and high-intensity statin therapy. Coronary angiography on hospital day 2 demonstrated 85% stenosis of the proximal left anterior descending artery and 60% stenosis of the right coronary artery. A drug-eluting stent was successfully placed in the LAD with no procedural complications. Post-procedure troponin trended down from 0.42 to 0.08 ng/mL over 48 hours. The patient remained hemodynamically stable and was transferred to the medical telemetry floor on hospital day 3. His diabetes was managed with insulin glargine 18 units at bedtime and a sliding scale of insulin lispro; fasting glucose values ranged from 142 to 178 mg/dL during the stay. He completed bedside cardiac rehabilitation evaluation on day 5.

PHYSICAL EXAM AT DISCHARGE

Vital signs: Temperature 36.8 C, heart rate 72 bpm, blood pressure 124/78 mmHg, respiratory rate 16, oxygen saturation 98% on room air. General: well-appearing, no acute distress. Cardiovascular: regular rate and rhythm, no murmurs, rubs, or gallops; right radial puncture site clean, dry, intact. Lungs: clear to auscultation bilaterally. Abdomen: soft, non-tender, non-distended. Extremities: no edema. Neurological: alert and oriented to person, place, time; cranial nerves intact.

KEY LABORATORY RESULTS AT DISCHARGE (2026-04-18)

- Hemoglobin: 13.8 g/dL (reference 13.5 to 17.5)
- White blood cell count: $7.9 \times 10^9/L$ (reference 4.0 to 11.0)
- Platelet count: $232 \times 10^9/L$ (reference 150 to 400)
- Sodium: 138 mEq/L
- Potassium: 4.2 mEq/L
- Creatinine: 1.1 mg/dL, eGFR 76
- Fasting glucose: 156 mg/dL (elevated; reference 70 to 99)
- HbA1c: 8.4% (elevated; target less than 7.0)
- Total cholesterol: 198 mg/dL
- LDL cholesterol: 122 mg/dL (elevated; target less than 70 for established CAD)
- HDL cholesterol: 38 mg/dL (low; target greater than 40)
- Troponin I: 0.06 ng/mL (down-trending; peak was 0.42)

DISCHARGE DIAGNOSES

1. Non-ST-elevation myocardial infarction (NSTEMI), status post percutaneous coronary intervention with drug-eluting stent to the left anterior descending artery.
2. Two-vessel coronary artery disease.
3. Type 2 diabetes mellitus, uncontrolled (HbA1c 8.4%).
4. Essential hypertension, controlled at discharge.
5. Hyperlipidemia, requiring intensification of lipid-lowering therapy.
6. Obesity (BMI 31.2), counseled on dietary modification.

DISCHARGE MEDICATIONS

- Aspirin 81 mg PO daily, indefinitely
- Clopidogrel 75 mg PO daily for 12 months post-stent, then re-evaluate
- Atorvastatin 80 mg PO nightly (increased from 40 mg for secondary prevention)
- Metoprolol succinate 25 mg PO daily (new, for cardioprotection)
- Lisinopril 20 mg PO daily (continue)
- Metformin 1000 mg PO twice daily (continue)
- Insulin glargine 18 units subcutaneously at bedtime (new, for glycemic control)
- Nitroglycerin 0.4 mg sublingual as needed for chest pain (use up to three doses, then call 911 if pain persists)

DISCHARGE INSTRUCTIONS

Activity: gradual return to normal activity. No lifting greater than 10 pounds for 5 days post-procedure. Walking encouraged. No driving for 48 hours. Diet: heart-healthy, low sodium (less than 2 grams daily), low saturated fat. Carbohydrate counting for diabetes management; dietitian referral provided. Smoking cessation: continue abstinence. Monitor: daily weights, blood pressure twice daily, fasting glucose four times daily. Return to the emergency department immediately for chest pain, shortness of breath, palpitations, syncope, or any bleeding from the radial puncture site.

FOLLOW-UP APPOINTMENTS

1. Cardiology clinic with Dr. Patel in 7 days (2026-04-25). Pre-visit labs: CBC, basic metabolic panel, lipid panel, HbA1c.
2. Primary care with Dr. Chen in 2 weeks (2026-05-02).
3. Cardiac rehabilitation enrollment within 2 weeks; first session scheduled 2026-04-29.
4. Diabetes educator and dietitian consultation in 10 days.

CONDITION AT DISCHARGE

Stable. Ambulating without assistance. Tolerating regular diet. Pain free at rest. Patient and spouse verbalize understanding of medications, warning signs, and follow-up plan.

Electronically signed by Sarah Chen, MD on 2026-04-18 at 14:22 EDT.